

NEW



2026

Ethiopian Health Profession Licensing Examination(EHPLE)

INFORMATION BOOKLET

EMERGENCY AND CRITICAL CARE NURSING



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MINISTRY OF HEALTH-ETHIOPIA

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HEALTHIER CITIZENS FOR PROSPEROUS NATION



AAU-Testing Center

Message From the State Minister, Ministry of Health -Ethiopia



H.E Mrs. Frehiwot Abebe

Improving healthcare quality is a global priority for sustainable development, with high quality healthcare being a key component of universal health coverage. One strategy to maintain health care standards is through provision of health professionals' competency assessment. Consequently, in 2019, the Ministry of Health-Ethiopia, initiated the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in seven health disciplines, which has since expanded to include 17 health disciplines.

The main goal of this competency assessment is to identify health professionals with minimal competencies necessary to perform their duties safely and competently, thus enhancing the quality of health care services. This initiative is overseen by a dedicated Health and Health Related Institutions and Professionals' Regulatory Lead Executive Office (LEO), comprising four desks, which plays a pivotal role in strengthening the system and enabling the LEO to conduct the competency exam more extensively and with improved organization and quality.

It is important to note that this competency assessment differs significantly from traditional academic or employment examinations. Hence, this information booklet has been created to address the informational needs of both examinees and teaching faculty regarding the EHPLE. Additionally, it aims to facilitate the assessment process, while promoting transparency and ensuring the sustainability of the program.

The preparation of this booklet involved the collaboration of esteemed experts from various higher education institutions, AAU-IER, the Ministry of Health, JHPIEGO-Ethiopia, Amref, Health Professionals 'Associations, and the Ministry of Education. Their invaluable contributions are acknowledged with sincere gratitude, alongside appreciation for the Ministry of Health staff for their unwavering commitment and hard work throughout the project.

Acknowledgements

This Information Booklet for EHPLE is a contribution from several educators, researchers, students and concerned individuals with a genuine interest to propel Ethiopia's medical and health sciences education forward.

The Ministry of Health is grateful for the contribution of many individuals and institutions in realizing this endeavor. Among these are Professional Associations, Student Association, Higher Education Institutions (both public and private), JHPIEGO-Ethiopia, AMREF/HWIP, MoE (Ethernet), UNFPA, AAU-IER and all HHRIPR LEOs staff.



Acronyms and Abbreviations

EHPLE	Ethiopian Health Profession Licensing Examination
ETA	Educational and Training Authority
HEIs	Higher Education Institutions
HHrIPR-LEO	Health and Health-related Institutions and Professionals Regulatory Lead Executive Office
HSTP-II	Health Sector Transformational Plan-II
MCQ	Multiple Choice Question
MOH	Ministry of Health
WHO	World Health Organization

Purpose of the Information Booklet

The EHPLE Information Booklet serves as a comprehensive guide for those individuals seeking information about the exam. It typically outlines basic information for candidate registration, exam development and administration processes and procedures, result notification, and the licensing process. It also includes information on the exam framework, i.e., the exam domain, sub-domain, content, process, and task, with sample exam items specific to each profession.

The publication of this Booklet is crucial for the following reasons:

- **Clarity and guidance:** It provides clear information about the exam by ensuring candidates understand the necessary information to prepare them.
- **Accessibility:** It serves as a readily accessible resource for individuals pursuing to take the exam, consolidating essential information in one document and facilitating easy access to necessary details. It also helps other stakeholders who might be interested in such resources.
- **Transparency:** It promotes transparency in the examination process and fosters trust among stakeholders about the exam.

In summary, the publication of this Booklet is essential for creating a transparent, standardized, and accessible framework that guides candidates through the EHPLE process.

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Definition of terms

- **Domain:** a broad category or area of knowledge or skills of a profession
- **Sub-domain:** a subset of a broader domain that focuses on knowledge or skills related to the overarching domain
- **Content:** a more specific subcategory, which is a breakdown of the sub-domain
- **Task:** the responsibility, knowledge, skill, and attitude of a junior undergraduate professional in an actual work environment
- **Process:** a systematic sequence of steps or actions designed to achieve a specific outcome
- **Learning outcome:** a clear and measurable statement that describes what the examinee is expected to know or be able to do
- **Relative emphasis:** the proportional importance or weight assigned to different content areas or categories within the assessment
- **Item:** a particular multiple-choice question
- **Item developer:** a subject matter expert responsible for writing test items or questions that make up the examination
- **Item reviewer:** a subject matter expert responsible for reviewing and refining the test items or questions that make up the examination
- **Standard setting:** a process of determining a cut-off point or passing score for an exam
- **Item difficulty index:** a statistical measure that indicates the proportion of examinees who answered a particular test item correctly
- **Discrimination index:** a statistical measure that evaluates how well a particular test item differentiates between high-performing and low-performing examinees
- **Admission paper:** a printout paper generated by the system after completing registration that contains the examinee's photo, QR code, and necessary information

1. Introduction

1.1. Background

Competency assessment is one of the strategies for controlling the standard of healthcare services provided in healthcare facilities. The World Health Organization (WHO) recommends all healthcare professionals to have necessary competencies. In Ethiopia, the Health Sector Transformational Plan-II (HSTP-II) states competency assessment of all graduates before joining the health workforce as one of the strategic initiatives.

The Ministry of Health (MOH) launched the Ethiopian Health Profession Licensing Examination (EHPLE) for undergraduates in 2019. The Health and Health-related Institutions and Professionals Regulatory Lead Executive Office (HHrIPR-LEO) of the Ministry of Health is given a mission to implement the Ministry's objective to achieve competency-related goals. It has the responsibility to ensure that the EHPLE meets technical, professional, and legal standards, and to protect the health, safety, and welfare of the public by assessing candidates' abilities to practice competently.

Currently, the exam is given for 17 health professions (Medicine, Nursing, Public Health, Pharmacy, Medical Laboratory Science, Anesthesia, Midwifery, Dental Medicine, Medical Radiology Technology, Environmental Health, Psychiatric Nursing, Pediatric and Child Health Nursing, Emergency and Critical Care Nursing, Human Nutrition, Surgical Nursing, Optometry, and Physiotherapy). Since its introduction until December 2025, a total of 204,811 examinees took the exam in 23 rounds.

1.2. The Rationale of EHPLE

One of the critical functions of the MoH is to guarantee the efficiency, quality, and equity of healthcare delivery and to protect the public from any undesirable consequences in healthcare delivery practices. As professionals' competence is a significant determinant of the quality of health, evaluation of health professionals' competence has now been given due attention. The licensing examination for health professionals serves as a crucial step to ensure that individuals entering the field meet specific competency standards. The sole aim of the competency assessment is to safeguard public health by verifying that health professionals have the minimal basic knowledge, attitude, and skill required to provide safe and effective care.

Licensing exams act as a preventive measure, ensuring that only competent professionals join the health workforce, which, in turn, contributes to reducing the occurrence of medical errors and enhancing overall patient safety. By setting standards through examinations, regulatory bodies strive to minimize the risk of medical errors caused by incompetence.

2. Key processes of EHPLE

EHPLE involves several key processes to ensure the quality and reliability of the examination.

2.1. Registration of candidates

EHPLE has a mandatory online registration system for both new and repeat candidates, which can be found at <http://hple.moh.gov.et>

Please note these important notes during registration.

New Test Takers:



- The list of eligible candidates from governmental and private Higher Education Institutions (HEIs) will be uploaded to MoH's online registration system by the registrar office of their respective HEIs.
- Candidates who completed their education abroad must bring the following documents in person to MoH in order for their names to be uploaded to the online registration system:
 - ✓ Equivalence document from ETA
 - ✓ Externship attachment completion letter after completing the externship assigned by the regulatory body
- Once the name of the candidate is uploaded to the system and registration has opened for the current exam round, the candidate must register at <http://hple.moh.gov.et> by uploading the necessary documents listed below.
 - ✓ a scanned original or temporary degree
 - ✓ a scanned national ID

Repeat Test Takers:



- Since the information about re-exam candidates already exists in the system, the candidate should register by directly going to <http://hple.moh.gov.et>. There is no need for re-exam candidates to upload their documents.

Both new and repeat candidates:



- After completing the registration, the candidate must download and print the admission paper by logging into his/her account using username and password, or national ID FAN.
- The candidate can change the exam center by logging into his/her account only during the registration period
- An application for center change is not allowed after the registration period is closed.

2.2. Task Analysis

The first step of exam development involves conducting a comprehensive task analysis study, which identifies the tasks, knowledge, skills, and abilities required from a junior undergraduate professional in the specific profession. The analysis is typically done through surveys, interviews, or observations of practitioners in the actual work environment, as well as through the Delphi method with subject matter experts.

2.3. Exam Blueprint

Based on the task analysis findings, the exam blueprint is prepared that outlines the content areas to be covered in the examination and the weight or emphasis given to each area. This ensures that the exam reflects the key competencies and knowledge needed for competent practice in that specific profession. Blueprint is the matrix or chart that shows the number and type of test questions represented across the topics in the content area, consistent with the learning outcome and relative weight of the test given to each content area. The blueprint also identifies the percentage weighting of cognitive dimensions as the level of competence tested in each knowledge domain.

Key components of blueprint are:

- Domain
- Sub-domain
- Content
- Task
- Process
- Learning outcome
- Assessment methods
- Assessment tools/instrument (test format)
- Relative emphasis (in percentage)

2.4. Item Development

The items are developed following specific guidelines to ensure clarity, relevance, and fairness. Subject matter experts with experience in the field are selected from HEIs to develop exam questions (items) that align with the exam blueprint. The exam questions focus mainly on “knows how” according to the competency level of the Miller's pyramid. The items are produced in a secure location on designated computers that are free from internet connectivity. The items are scenario-based and constructed with stem, lead-in, and four options/alternatives. All items will have a single-best-answer type of Multiple-Choice Question (MCQ) that addresses the learning outcome defined in each content area. Standard textbooks, updated guidelines, and standards are used as reference materials.

2.5. Item Review

Once developed, the items undergo a rigorous review process by item reviewers. The main purpose of the exam review process is to evaluate content relevance, technical accuracy, clarity, and sensitivity related to culture and religion. More experienced subject matter experts as well as psychometric experts will do the review to ensure the items meet psychometric standards. Subject matter experts shall review the items to confirm that they are accurate, clearly stated, and correctly keyed using the checklist. Psychometric experts shall review the items to ensure that

they are not technically flawed. They also work on editorial review to check grammar, punctuation, and spelling errors. This helps ensure the reliability and validity of the items.

2.6. Standard setting method

The standard setting or cut-off point of the EHPLE is determined using the Modified-Angoff method, which is one of the most widely used and legally defensible standard setting approaches to set a cut-off point for high-stake competency examinations.

The method involves a panel of subject matter experts who evaluate each exam question and then estimate the probability that a minimally competent examinee would answer each test item correctly. The average of the experts' predictions for an exam question becomes its predicted difficulty. The average of the predicted difficulty values across all items on a test is the recommended cut-off point. This point indicates the minimum level of knowledge and skill required to pass.

2.7. Exam Administration

The EHPLE is administered following established protocols and guidelines. Proper exam administration procedures, appropriate security measures, and appropriate consideration for test-takers who need special support will be applied during exam administration at exam centers. The exam is administered in selected HEIs nationally, where candidates can choose based on their convenience at the time of registration. The exam schedule will be posted ahead of time on the MoH website and social media pages. Examinees who have a valid admission paper are eligible to sit for the exam. The mode of exam administration is computer-based testing.

CAUTIONS

-
- Candidates are allowed



- Attend the orientation session in order to sit for the exam
 - Arrive at the exam center on time
 - Bring a legal ID and admission paper
 - Complete the exam within the allotted time frame
-

- Candidates are **NOT** allowed



- To bring reference materials, blank paper, or notes into the exam center
- To smoke, eat, or drink in the exam room
- To bring mobile phones, tablets, smart watches, camera devices, eyeglasses, calculators or any type of electronic device into the exam center
- To bring their personal belongings to the exam center
- To bring weapons and sharp materials into the exam center
- To give or receive assistance to or from other candidates during the examination

2.8. Scoring and post exam analysis

Once the exam is completed, the scoring process begins. The exam scoring process involves computerized scoring using software.

Post-exam analysis is the process of analyzing examinees' responses to individual test items in order to assess the quality of the items and the exam as a whole. This phase helps to identify any poorly performing items that may need revision or removal from the exam. The item difficulty index, discrimination index, and reliability coefficient are elements of exam analysis.

2.9. Result notification and appeal management

After scoring and analysis, individual score reports are generated and provided to examinees through the website <http://hple.moh.gov.et>. After result notification, examinees can submit their appeal through phone or email within 10 working days after result notification.

2.10. Licensing

The list of examinees who passed the exam will be sent to regional and city administration regulatory bodies. A license is obtained from the regional/zonal health bodies where he/she permanently lives.

Requirements for professional licensing are:



- Passing the EHPLE
- Original or temporary degree
- Educational documents (10th and 12th certificates)
- Medical certificate
- Government issued ID
- Additional prerequisites based on the requirements of regional regulatory bodies

3. Exam framework

The key broader professional roles, also known as domains or main knowledge areas serve as a building framework for the licensing examination content for Emergency and Critical care nurses. The domains are further divided into discrete professional attributes that constitute sub-units (also referred to as sub-domains) defining the professional identity for Emergency and Critical care nurses, specifying the performance level of each sub-domain serve as the final characteristic of the professional duties on which the licensing exam focuses.

The contents of the licensing examination are presented below, structured into key roles (domains), sub-units (sub-domains), and tasks. The examination emphasis for each domain and sub-domain, out of the total 100% questions, is indicated in brackets.

Key professional roles/ domains

- ☐ Patient Care (85.0%)
- ☐ Scholar (4.0%)
- ☐ Professionalism (4.0%)
- ☐ Leadership and management (3.0%)
- ☐ Health promotion and disease prevention (4.0%)

Key role/ domain 1: Patient care (85.0%)

Description: This domain encompasses the professional roles of Emergency and Critical care nurses in the provision of high-quality, safe, and patient-centered holistic nursing care within their scope of practice. The provision of up-to-date, ethical, and resource-efficient nursing care requires the application of integrated knowledge of biomedical, fundamental nursing, clinical, behavioral, and social sciences. This is conducted in collaboration with patients and their families, other healthcare professionals, and the community. As patient care providers, ECCN shall implement the Emergency and Critical nursing care process by collecting and interpreting information, making clinical decisions, carrying out diagnostic and therapeutic nursing interventions, providing nursing care, and evaluating interventions. To demonstrate competence in this domain candidates shall apply such integrated knowledge in the following sub-areas:

- ☐ Respiratory system (16.0%)
- ☐ Cardiovascular system (16.0%)
- ☐ Nervous system (6.0%)
- ☐ Gastrointestinal system (6.0%)
- ☐ Genitourinary system (5.0%)
- ☐ Endocrine system (4.0%)
- ☐ EENT system (3.0%)
- ☐ Immune system and Integumentary (3.0%)
- ☐ Trauma and musculoskeletal system (20.0%)
- ☐ Special Population (6.0%)

Key role/ domain 2: Scholar (4.0%)

Description: This domain encompasses the professional roles of Emergency and Critical care nurses in generating and utilizing scientific data to improve the health and well-being of Ethiopians and broaden their scientific knowledge within the healthcare system and community setting. Providing this service requires the application of integrated knowledge in research methods, measurements of health and disease, biostatistics, epidemiology, clinical audit, evidence-based practice, and research ethics. To demonstrate competence in this domain, candidates must possess applied knowledge in planning, problem identification, data collection, analysis, interpretation, report write up, and dissemination of research outputs.

Key role/ domain 3: Professionalism (4.0%)

Description: This domain encompasses the professional commitment of Emergency and Critical care nurses to promoting the health and well-being of individuals and society through adhering to ethical standards, maintaining personal integrity, and upholding high standards of competence in all areas of practice. To exhibit competence in this domain, candidates must possess applied knowledge of ethical principles, medico-legal practices, effective communication, accountability to the profession and society, maintenance of professional excellence and personal health, and professional values such as compassion, respect, integrity, honesty, altruism, and humility.

Key role/ domain 4: Leadership and management (3.0%)

Description: This domain encompasses the professional roles of Emergency and Critical care nurses in envisioning a high-quality healthcare system through self-awareness, active participation in healthcare teams, leading teams, and managing health systems. Providing this service requires the application of integrated knowledge in continuous quality improvement, effective health system leadership, management, and healthcare ethics. To demonstrate competence in this domain candidate shall possess applied knowledge to plan, organize, staff, lead, execute, monitor, and control healthcare resources and activities.

Key role/ domain 5: Health promotion and disease prevention (4.0%)

Description: This domain encompasses the professional roles of Emergency and Critical care nurses in enhancing the health and well-being of patients, communities, and the larger populations they serve through health advocacy, disease prevention, health promotion and the promotion of health equity. Providing this service takes an integrated understanding of determinants of health, health informatics, epidemiology, communicable disease control, health education.

Table 1 Exam Content for Emergency and Critical Care Nursing Profession

Domain: Patient care
Sub Domain 01: Respiratory System
Tasks
Assess patients with common respiratory emergency and critical problems
Diagnose patients with common respiratory emergency and critical problems
Formulate plan and manage patients with common respiratory emergency and critical problems
Monitor and evaluates the progress of emergency and acutely ill patients with respiratory problem
Sub Domain 02: Cardiovascular system
Tasks
Assess patients with common cardiovascular emergency and critical problems
Diagnose patients with common cardiovascular emergency and critical problems
Formulate plan and manage patients with common cardiovascular emergency and critical problems
Monitor and evaluate the progress of emergency and acutely ill patients with cardiovascular disorders
Sub Domain 03: Nervous system
Tasks
Assess patients with common nervous system emergency and critical problems
Diagnose patients with common Nervous system emergency and critical problems
Formulate plan and manage patients with neurological emergency and critical problems
Monitor and evaluate the progress of emergency and acutely ill patients with nervous system disorders
Sub Domain 04: GI System
Tasks
Assess patients with gastrointestinal emergencies and critical problems
Diagnose patients with common GI emergency and critical problems
Formulate plan and manage patients with GI emergency and critical problems
Monitor and evaluates the progress of emergency and acutely ill patients with GI system problems
Sub Domain 05: GU System
Tasks
Assess patients with Genito-urinary emergency and critical problems
Diagnose patients with Genito-urinary emergency and critical problems
Formulate plan and manage patients with GUT emergency and critical problems
Monitor and evaluate the progress of emergency and critical patients with GU system problems
Sub Domain 06: Endocrine system
Tasks
Assess patients with endocrine emergency and critical problems
Diagnose patients with endocrine emergency and critical problems
Formulate plan and manage patients with common endocrine emergency and critical problems
Monitor and evaluates the progress of emergency and acutely ill patients with endocrine problems

Sub Domain 07: EENT system
Tasks
Assess and diagnose patients with emergency and critical EENT Problems
Formulate plan, manage and evaluate patients with EENT Emergency Problems
Sub Domain 08: Immune system and Integumentary
Tasks
Assess and diagnose common immunologic and integumentary emergencies
Formulate plan, manage and evaluate patients with common immunologic and integumentary emergencies
Sub Domain 09: Trauma and musculoskeletal system
Tasks
Assess patients with common traumatic emergency and critical problems.
Diagnose patients with common traumatic emergency and critical problems.
Formulate plan and manage patients with traumatic emergency and critical problems
Monitor and evaluates the progress of emergency and critical patients with trauma (Mental status, Vital signs, Oxygenation, Ventilation, hemodynamic, ICP)
Sub Domain 10: Special Population
Content
Common gynecologic and obstetric emergency and critical problems (APH, PPH, Abnormal Uterine Bleeding (AUB), Preeclampsia and Eclampsia, Puerperal Sepsis and Active third stage labour)
Common Pediatric and neonatal emergencies (Acute meningitis, Dehydration, Severe pneumonia, hypoglycemia, Asphyxia, Febrile seizure, Neonatal sepsis and Hypothermia)
Common Psychiatric emergencies (Acute psychosis, Suicide attempt and Delirium)
Domain: Professionalism
Tasks
Adhere to standard of practice (Standards, ethical principles, roles, ethical dilemma, Bill of rights, Consent, MRC)
Produce and maintain effective communication, documentation and Reporting
Domain: Scholar
Tasks
Develop research proposal, analyze quantitative data and calculate measures of disease morbidity and mortality
Domain: Leadership and Management
Tasks
Manage and lead health care system
Implement emergency and critical care service quality audits
Domain: Health Promotion and Disease Prevention
Tasks
Assess community health emergencies (surveillance and screening for emerging and re-emerging issues)
Diagnose and intervene public health emergencies

Sample Questions

1. A 55-year-old female patient, who has a respiratory distress and diagnosed with acute severe asthma, is admitted to an emergency department. She also has productive cough and chest tightness. Her physical examination reveals: respiratory rate of 29/min, pulse rate of 120/min and oxygen saturation of 91 percent.

What is the most appropriate nursing care plan for this patient?

- (A) To position the patient to upright position
- (B) To position the patient to supine position
- (C) To position the patient to prone position
- (D) To administer pain relief medication

Answer key: The answer is **A**

Answer key: The answer is **D**

Explanation: In this case scenario, the patient is diagnosed with moderate asthma with shortness of breath, cough and chest tightness. Option D is the priority nursing intervention to relieve the patient from difficulty of breathing by providing high flow volume of oxygen. Consulting physician for intubation (option A) can be considered as the priority nursing intervention but it is not the priority intervention as intubation is the last intervention option. Securing IV line with normal saline (option B) can be the alternative option to maintain fluid balance but it is not the priority nursing intervention for relieving shortness of breath. Positioning the patient on flat position (option C) may

be considered as an alternative intervention but it cannot adequately relieve shortness of breath.

3. A 50-year-old female patient came to an emergency room with complaints of frequent vomiting and diarrhea. She informed that the diarrhea is painless and lasted for more than 24 hours. On assessment, she is lethargic, has scaphoid abdomen, and poorly returned skin turgor with cyanotic appearance.

What is the most appropriate initial assessment for this patient?

- | | |
|-------------------------------------|-------------------------------------|
| (A) Rapid toxicology screening | (C) Quick bed side ultrasonography |
| (B) Urgent laboratory investigation | (D) Rapid assessment of vital signs |

Answer key: The answer is **D**

Explanation: In this case scenario, the patient complains of frequent vomiting and diarrhea which lasts more than 24 hours. Rapid assessment of vital signs (option D) is the most appropriate initial assessment to identify status of vital signs of a patient which helps to determine the diagnosis and provide clue for the management. Rapid toxicology screening (option A) can be used as alternative assessment but is not the first step assessment for this patient. Urgent laboratory investigation (option B) can be considered as alternative assessment method but it is done after the initial assessment. Quick bed side ultrasonography (option C) can be used as an assessment technique to diagnose the patient's problem but it is requested based on initial assessment and laboratory findings.

4. A 42-year-old male patient is admitted to an emergency department with a diagnosis of acute coronary syndrome. He complains of severe chest pain that radiates to his shoulder. On assessment, the nurse found that he is anxious and restless. His vital signs show: Blood pressure of 80/60 mmHg, Pulse rate of 55/min, Respiratory rate of 28/min and temperature of 37.5 °C.

What is the most appropriate initial care plan for this patient?

- | | |
|---|--|
| (A) To restore damaged myocardial cells | (C) To relieve pain and ischemic signs |
| (B) To reduce anxiety and restlessness | (D) To eliminate etiologic factors |

Answer key: The answer is **C**

Explanation: In this case scenario, the patient is complaining of severe chest pain with unstable vital sign. Relieving pain and ischemic sign (option C) is the most appropriate initial care plan which helps to relieve patient from severe chest pain and stop further tissue ischemia and increases tissue perfusion to the area. To restore damaged myocardial cells (option A) can be considered as alternative nursing care plan but it is not immediate care plan for this patient. To reduce anxiety and restlessness (option B) can be used as alternative care plan but this can be provided after patient relieve from severe pain and ischemia. To eliminate etiologic factors (option D) can be used as alternative care plan

but it cannot be effective initial care plan for a patient with in severe chest pain and developed ischemia

5. A 24-year-old female patient, who is a known epileptic for the last eight months, was brought to an emergency room after she suffered from a recurrent seizure for 20 minutes. On assessment, she has respiratory rate of 28/min, blood pressure of 150/55 mm Hg, and pulse rate of 59/min. She is using her neck muscle for breathing and chest in drawing is observed. Oxygen saturation of 89 percent is recorded.

What is the most appropriate definitive management for the patient?

- | | |
|-----------------------------|---------------------------|
| (A) Positioning the patient | (C) Using paralytic agent |
| (B) Intubating the patient | (D) Sedating the patient |

Answer key: The answer is **B**

Explanation: In this scenario, the patient is in a recurrent seizure for 20 minutes with altered vital signs. Intubating the patient (option B) with recurrent seizure helps to provide brain oxygenation and to secure airway. Positioning the patient (option A) can improve oxygenation but it is not appropriate definitive management for a patient with recurrent seizure with unstable vital signs. Using paralytic agent (option C) can be considered as alternative management for this condition but it is not definitive management for a patient with recurrent seizure. Sedating the patient (option D) can be used as alternative management but it is not the most appropriate definite management for a patient with unstable vital signs.

6. A 79-year-old male patient is presented to an emergency room due to persistent headache secondary to brain tumor. His follow-up is at oncology department. The patient reported that his physician has told him that his tumor cannot be removed by surgery. On assessment, his general appearance is sick looking, GCS is 12/15, blood pressure is 140/60 mmHg, pulse rate is 60/min, respiratory rate is 16/min, temperature is 37.8 °C, and blood oxygen saturation is 96 percent. Blood electrolyte result shows Na⁺ level of 139mmol/L, K⁺ level of 3.5mmol/L, Cl⁻ level of 90mmol/L, and HCO₂ level of 20mmol/L.

What is the most appropriate management option for the patient?

- | | | | |
|-----------------|---------------|--------------|------------|
| (A) Antibiotics | (B) Radiation | (C) Hormonal | (D) Biopsy |
|-----------------|---------------|--------------|------------|

Answer key: The answer is **B**

Explanation: In this scenario, the patient is in persistent headache secondary to brain tumor which cannot be removed by surgery. Radiation (option B) therapy is used to interrupt or eliminate the growth of malignant cells. Antibiotics (option A) can be provided for a patient with brain tumor to prevent infection that caused due to immune compromising but it cannot be appropriate management to destroy tumor. Hormonal (option C) can be considered as alternative management but cannot be effective to

eliminate the tumor. Biopsy (option D) is a diagnostic procedure to remove a small sample of tissue to be examined microscopically to detect malignant cells but it is not appropriate management to avoid the tumor.

7. A 45-year-old male patient comes to an emergency room with complaints of constipation, acute abdominal pain, abdominal distension and vomiting of three days duration. Physical examination reveals the presence of tachycardia, dehydration, and abdominal rigidity.

What is the most relevant investigation to diagnose this case?

- | | |
|-----------------|-----------------------|
| (A) CT-scan | (C) Urine analysis |
| (B) Chest X-ray | (D) Stool examination |

Answer key: The answer is **A**

Explanations: In this scenario, CT scan is the most widely used diagnostic method for patient with signs and symptoms of gastrointestinal disorders. CT scan (option A) is more accurate diagnostic tool to identify abdominal pathology, which also provides detail information about the abdomen. If there is an intestinal obstruction, CT scan can identify the location, severity, and the causes of obstructions. CT scan result will also guide which type of management is appropriate for this patient. Chest X-ray (option B) is may be considered as a diagnostic tool for respiratory disease. Urine analysis (option C) can be used to diagnose any disease associated with genitourinary system and stool examination (option D) is used to identify parasitic disease. In this case the three options are not mostly used diagnostic methods for patient with signs and symptoms of gastrointestinal disorders.

8. A 25-year-old male patient came to a health center emergency room with complaints of excessive sweating, increased urination and vomiting of six hours duration. On assessment, he is lean, has pulse rate of 140/min, respiratory rate of 30/min and hypothermia. Laboratory finding shows that he has urine ketones+3 and plasma glucose level of 300mg/dl.

What is the most likely diagnosis of the patient?

- | | |
|--------------------|---------------------------|
| (A) Adrenal crisis | (C) Hypoglycemia |
| (B) Hyperglycemia | (D) Diabetic ketoacidosis |

Answer key: The answer is **D**

Explanations: In this scenario, the patient is presented with a typical clinical presentation of diabetic ketoacidosis. DKA (option D) is an emergency condition which is manifested by the three clinical features such as hyperglycemia, dehydration and acidosis with electrolyte loss. In addition, a patient with DKA has polyuria, polydipsia, hypotension, nausea and vomiting. Adrenal crisis (option A) is a life-threatening clinical condition due to insufficient levels of cortisol. The clinical manifestations include: hypotension, abdominal pain, nausea and disorientation. Ketone is not positive on patient with

adrenalin crisis and hypoglycemia. Hyperglycemia (option B) alone may cause long term complication on the patient, which is not acute and fatal like DKA. Hypoglycemia (option C) is a common complication with diabetic mellitus patient. A patient with hypoglycemia is manifested by low plasma level of blood glucose, change in mental status and negative ketones.

9. A 35-year-old male patient is referred to a surgical emergency ward with a diagnosis of septic arthritis. Even though he has been taking broad spectrum IV antibiotics, he showed no improvement. A nurse assessed the patient and diagnosed the case as deficient knowledge about septic arthritis process and treatment regimen.

What is the most important nursing intervention for the patient?

- (A) Improving the bowel elimination
- (B) Controlling the infectious process
- (C) Supporting adherence of prescribed antibiotics
- (D) Observing weight-bearing and activity restriction

Answer key: The answer is C

Explanations: In this scenario, the patient would be treated with broad spectrum antibiotics. Once antibiotic is started, the nurse should support the patient for adherence of antibiotics. The nurse should counsel and advise the patient about the importance of treatment adherence and associated clinical outcome. Improving the bowel elimination (option A) is an important nursing intervention for all patients who are admitted to a ward but in this case there is no association between bowel elimination and response to treatment. Controlling the infection process (option B), indicates that the nursing diagnosis is knowledge deficit, the infection process will be controlled when broad spectrum antibiotics is administered properly and when good adherence is ensured. Observing weight-bearing and activity restriction (option D), may be applied to prevent further complications, but this kind of nursing interventions doesn't have a direct association with knowledge deficit, drug adherence and response to treatment.

10. A 33-year-old male patient was referred to an emergency department from district health center with a diagnosis of femoral and pelvic fractures. During assessment, he complains of severe pain and petechiae in the buccal membranes and conjunctival sacs on the hard palate, the chest and anterior axillary folds. His blood pressure is 80/50 mmHg, temperature is 39.5 degree centigrade, SpO₂ is 80 percent, and respiratory rate is 24/min.

What is the most likely complication in this scenario?

- (A) Fat embolism
- (B) Heterotopic ossification
- (C) Compartment syndrome
- (D) Complex regional pain syndrome

Answer key: The answer is **A**

Explanation: In this scenario, fat embolism is the most common and early complication of patient with long bone and pelvic bone fracture. Usually, fat embolism occurs within 12-48 hours following injury and more common in adults younger than 49 years. Common clinical presentation of fat embolism is hypoxia, tachypnea, tachycardia, chest pain, wheezing and cough. Heterotopic ossification (option B) is a complication which is manifested after bone fracture. It is an abnormal bone formation which occurs near to bone or within the muscle. Clinical manifestations are pain over the affected muscle, and restricted range of motions. Compartment syndrome (option C) is one of limb-threatening conditions as a result of low tissue perfusions which lead to limb ischemia. Patient with compartment syndrome complains deep throbbing pain which is not respond to opioid treatment. Complex regional pain syndrome (option D) is a delay type of complication which usually occurs after fracture of upper extremity. It has clinical presentations of burning sensations, edema, stiffness and hyperesthesia.

11. A nurse is caring for a 62-year-old male patient diagnosed with delirium. The patient has no history of chronic illness, trauma, and substance abuse. Besides, he has no evidence of infection..

What is the most relevant history the nurse anticipates from this scenario?

- | | |
|----------------------------|---|
| (A) Systemic infections | (C) Hypertensive encephalopathy |
| (B) Substance-Intoxication | (D) Disturbance in the sleep–wake cycle |

Answer key: The answer is **D**

Explanations: In this scenario, delirium is characterized by abnormal level of consciousness and a change of cognitions which occurs rapidly within a short period of time. For a patient with delirium, it is very important to collect a relevant history of fluctuation between hypersomnolence and insomnia. The nurse should also collect a history of the sleep-awake cycle which is the most important. Systemic infection (option A) may have a direct relationship with delirium; in this case there is no sign of infection. Substance intoxication (option B) may be considered as an etiology for delirium, but in this scenario there is no suggestive sign of intoxications induced delirium. Hypertensive encephalopathy (option C), this is one of a medical condition which may result delirium. In this case there no signs of an increased blood pressure. Generally, a patient with delirium due to any causes the most important and relevant history that the nurse should collect is Disturbance in the sleep–wake cycle.

12. A district health office with the help of other stakeholders is responsible for displaced community members who are displaced due to drought. Hence, the office is currently trying to restore power, provide shelters, job assistance, critical stress debriefing for emergency responders and victims. It is also providing small business loans to enable the affected community regain a proper level of functioning in a short and long term bases.

What is the most likely disaster management phase applied in this case?

- (A) Preparedness (B) Mitigation (C) Prevention (D) Recovery

Answer key: The answer is **D**

Explanations: - In this scenario, Recovery phase is an essential component of disaster management. Recovery aims on long term activities which support the community by reconstructing of the physical infrastructure, providing emotional, social and physical support. This phase also facilitate the improvement post disaster crises. The recovery phase is long-lasting and which needs multisectoral involvement. Recovery involves rebuilding homes, replacing property, resuming employment, restoring business, and permanently repairing and rebuilding infrastructures. Preparedness (option A) is a time for preparations and makes our self-ready to respond to a disaster, to any crises and in any emergency situations. Mitigation (option B) usually it is an initial phase of disaster management which is designed to reduce further damage and losses. Prevention (option C) specifically this phase will focus on prevention of disaster before causing crises, and damage on human being and resources. An adequate preparation will also reduce severe damage.

13. A nurse working in an emergency room was providing care to a patient who has a stick injury on his head due to fighting. Since, the case is medico-legal issue; the nurse recorded the type of injury, date and time, name of the patient, cause of the injury and the treatment given for the patient.

What the most relevant information missing from the above scenario?

- (A) Ethnicity of the patient (C) Religion of the patient
(B) The size of the wound (D) Duration of the injury

Answer key: The answer is **B**

Explanation: In routine documentation of any emergency trauma documentation, it is mandatory to include critical components that would help to give clear picture of the injury in an objective manner. These includes type of injury, date and time, name of the patient, cause of injury and the treatment given. Regarding the wound, the site, the depth, and size are critical; hence, should document. In this scenario, the size of the wound was not mentioned while it is determinant to give final decision in court. Other factors like ethnicity and religion cannot be changed through time and have less effect on the court decision.

14. The head of a health center has organized a health development army team for a surrounding community to prevent home delivery and associated neonatal mortality. The team members of the health development army know what and how of performing their expected activities in a most effective manner.

What are the most likely features that characterize the team member in this case?

- (A) Having a synergy team
- (B) Having a shared decision making
- (C) Having a defined roles and responsibilities
- (D) Having a common focus between team members

Answer key: The answer is C

Explanation: Teams that are effective can achieve more together than they would as individuals all working alone. Awareness of the characteristics of a team helps to manage effectively the group. Effective teams are built on communication, trust, shared decision-making, positive reinforcement, cooperation, flexibility, focus on common goals, having well defined roles and responsibilities, and they should have synergy. In this scenario among these characteristics of effective team, the team members of the health development army know the expected activities they have to perform and how to perform those activities these indicates the defined roles and responsibilities of team members is clear.

15. A researcher planned to conduct a study on the risk factors of birth asphyxia among newborns. Before deciding on the topic, the researcher reviewed the results of previously conducted researches on the field to explore whether there are major questions that require further investigation.

What is the most likely criterion applied by the researcher to select the research topic?

- (A) Feasibility
- (B) Relevance
- (C) Political acceptability
- (D) Avoidance of duplication

Answer key: the answer is D

Explanation: Avoiding duplication is the method that the researcher investigates whether the topic has been researched and investigates major questions that deserve further investigation remain unanswered. The scenario tells us that the researcher has reviewed the results of previously conducted research on the field to explore the major questions that deserve further investigation to avoid duplication of works. However, choice A considers the complexity of the problem and the resources that the researcher requires for the study; Choice B is about how a research result is useful in policy formulation; choice C is about the interest and support of the authorities.

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